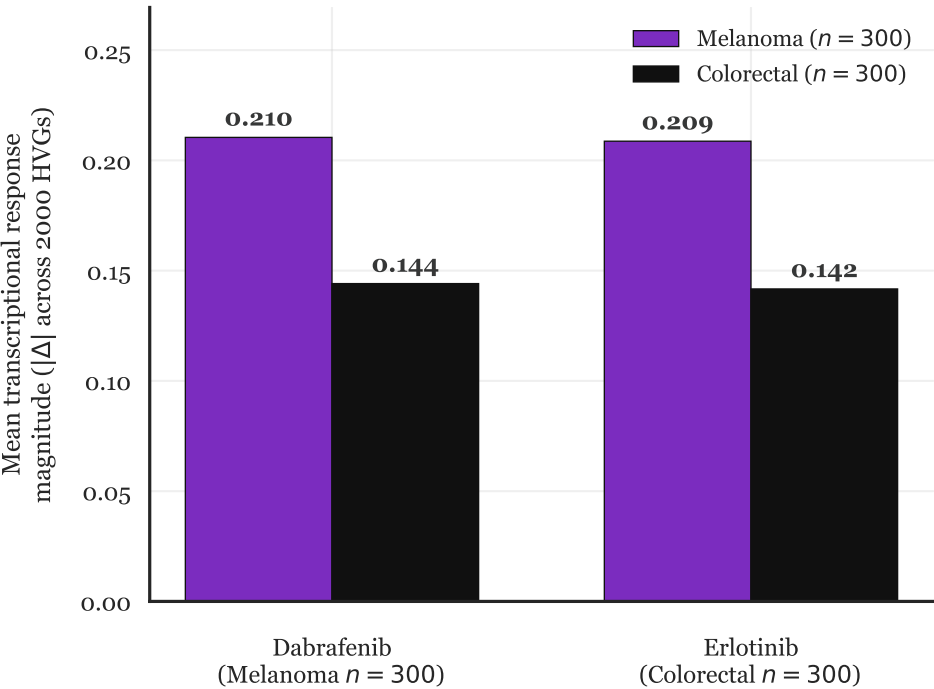
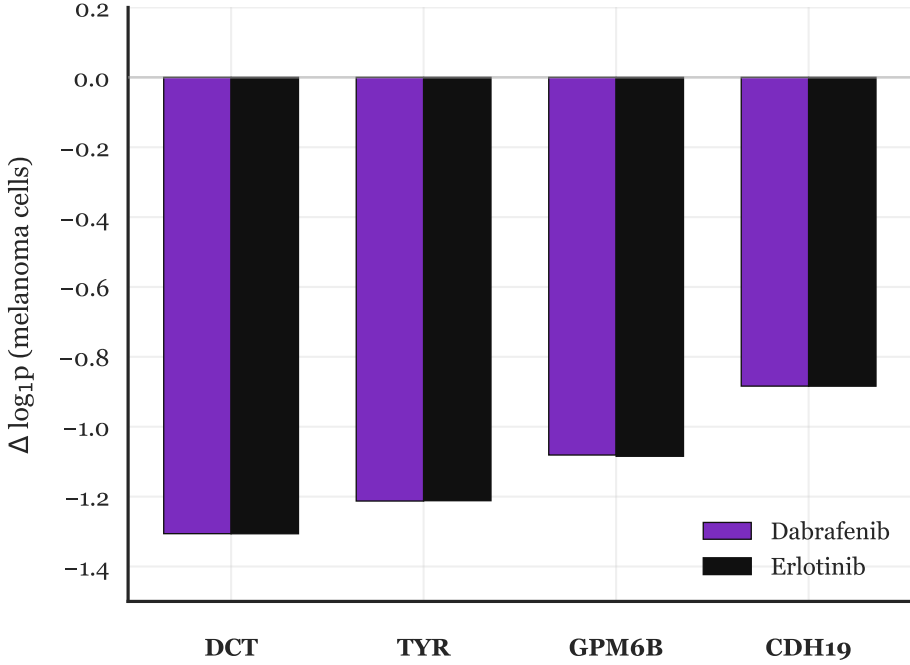


What STATE predicts: Dabrafenib (BRAF inh.) vs. Erlotinib (EGFR inh.)
across BRAF V600E melanoma and colorectal cancer

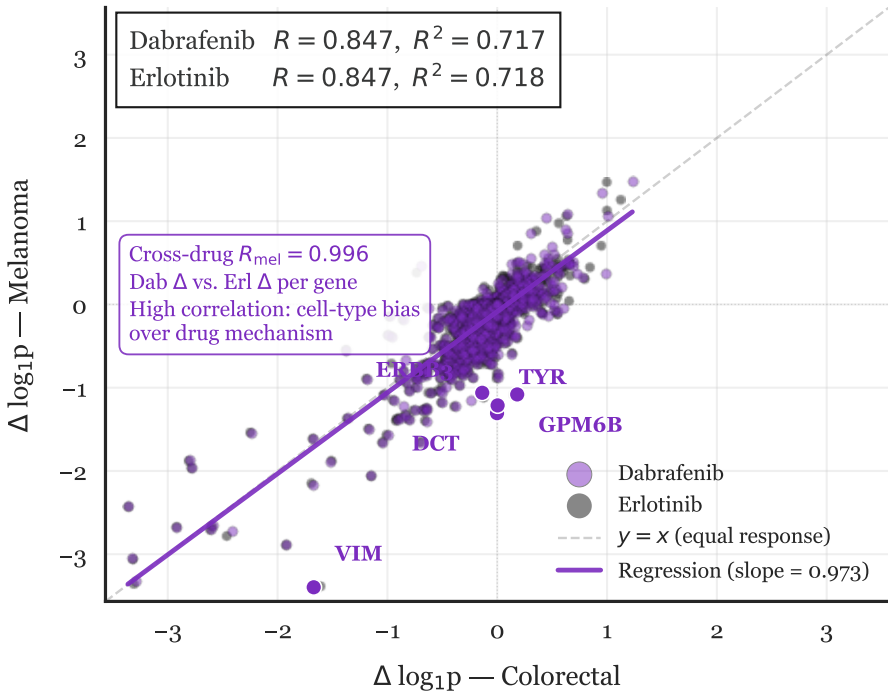
Overall transcriptional
response magnitude



Melanocyte identity genes
(melanoma cells only)



Per-gene Δ : melanoma vs. CRC
(all 2000 HVGs)



Panel 1: Mean absolute log-normalized delta ($|\Delta|$) across 2000 HVGs. Both drugs predict $\sim 1.46\times$ more response in melanoma than colorectal (Erlotinib: $1.47\times$) — a cell-type effect, not drug specificity. Panel 2: Predicted suppression of melanocyte identity genes is near-identical for Dabrafenib and Erlotinib (approaching or exceeding a log unit for DCT, TYR, GPM6B; ~ 0.9 for CDH19). Panel 3: Per-gene Δ correlation $R \approx 0.85$ for both drugs; cross-drug $R = 0.996$ confirms STATE cannot distinguish the approved BRAF inhibitor from an unrelated EGFR inhibitor.